

Look inside the anterior chamber

Pain and photophobia with anterior chamber leukocytes indicate more than uncomplicated conjunctivitis.

Decode the findings

Finding	Clinical meaning
Cells	Leukocytes floating in aqueous establish anterior inflammation
Flare	Protein makes the slit-lamp beam appear smoky
Keratic precipitates	Inflammatory deposits on corneal endothelium
Posterior synechiae	Inflamed iris adheres to the lens

Assess pattern and associations

AAU commonly produces a painful red eye, photophobia and blur with little discharge. Barrier disruption explains both cells and flare. Ask about recurrence, inflammatory back pain, joint symptoms, rash and urinary symptoms. The spondyloarthropathies associated with HLA-B27 include psoriatic, ankylosing, inflammatory-bowel and reactive disease. Systemic clues guide targeted investigation; infection such as syphilis or TB may also need consideration according to the presentation.

Control inflammation and prevent adhesions

Treatment combines anti-inflammatory care with relief of ciliary spasm and follow-up for complications.

Each treatment has a purpose

Topical steroid

Controls anterior inflammation; taper under supervision to reduce recurrence.

Cycloplegic

Relaxes ciliary spasm to relieve pain and dilates the pupil to help prevent synechiae.

Follow-up

Monitor inflammation, pressure and lens changes; co-manage a systemic association when present.

Do not treat symptoms alone

The clinical diagnosis is confirmed on slit lamp, not just by redness. Check for a corneal infection that may have spread to the anterior segment, because cause-specific therapy remains necessary. Steroid complications do not remove the need to treat inflammation; they make monitoring essential. Cycloplegia is an adjunct, not a substitute for inflammatory control. Persistent blur should trigger assessment of cornea, lens and macula rather than an automatic increase in drops.